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Greenway eCR / Version 1.0

User Guide

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Last Modified: 9/20/2024

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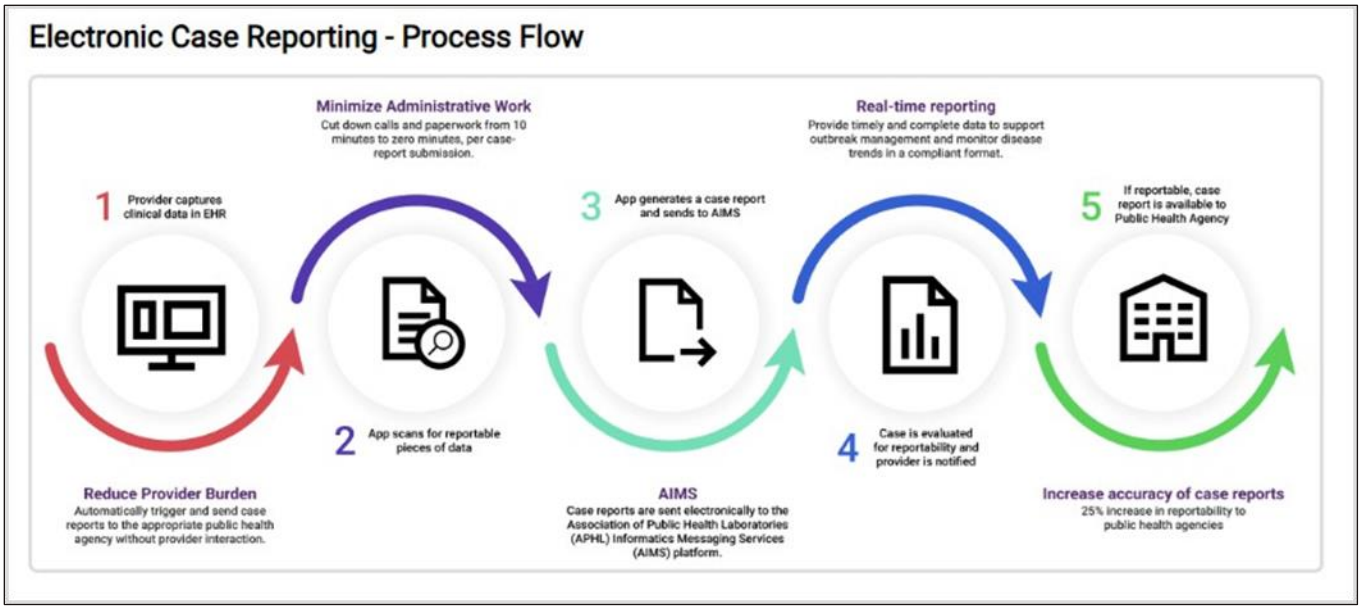
Electronic Case Reporting (eCR) User Guide

Section 1 - Benefits and high-level information

Section 1.1 - Introduction to Electronic Case Reporting

Electronic Case Reporting (eCR) is the automated generation and transmission of case reports from electronic health records to public health agencies for review and action. Automating case reporting reduces provider burden and minimizes follow-up investigation phone calls and paperwork. It also provides information to healthcare providers from public health about an identified reportable condition that can include notices of disease outbreaks, infection control information, and next-step testing and patient management. To purchase Electronic Case Reporting, visit MyGreenway for self-service purchase, or contact your Sales Representative.



Section 1.2 - eCR Benefits

Electronic Case Reporting (eCR) can help health agencies to:

- Track and monitor disease outbreaks, allowing them to identify potential health risks, intervene early, and reduce the spread of infectious diseases.
- Identify trends in healthcare utilization and access, helping to improve health outcomes for under-served populations.
- Collect and analyze data more quickly, allowing them to make more informed decisions about public health policy.

eCR replaces manual case reports that are generally done by mail, phone, fax, or through an online portal.

For Healthcare Providers

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- Saves time by eliminating manual data entry and reporting.
- Streamlines reporting to multiple jurisdictions.
- Fulfills the Centers for Medicare and Medicaid Services (CMS) program that promotes inter-operability requirements for eCR.
- Can fulfill legal reporting requirements.
- Can be implemented for all reportable conditions.

For Public Health Agencies

- Enables bidirectional data exchange.
- Provides more complete data to support outbreak management.
- Efficiently monitors the spread of reportable diseases.
- Reduces response time with automated information.
- Supports submission of case-based data (without identifiable information) to the Centers for Disease Control and Prevention (CDC) through the National Notifiable Diseases Surveillance System.

Section 1.3 - The Case Reporting Process

Manual case reporting process

- A provider starts a patient visit and documents the patient's health information into the EHR.
- The provider may have to send "reportable" health info to their local Public Health Agency (PHA).
- The health info is reported manually to the PHA as a "case report" by sending a fax, e-mail, or making a phone call.
- It takes an avg. of ~10 minutes per case report to document, send, and follow up on the required information.

Electronic case reporting process

- eCR automatically pulls health information from the patient's chart when a "reportable" piece of data is documented.
- eCR will create and send a "case report" to the Association of Public Health Laboratories Informatics Messaging Services (AIMS) platform, where the report is reviewed for "report-ability".
- A "reportability response" is sent back to the provider with the status of the case report's reportability.
- If "reportable", the case report is automatically made available to the practice's local Public Health Agency.

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- Healthcare providers can now easily create and send electronic case reports during patient visits, saving time and improving accuracy by 25%. This streamlined process allows professionals to focus on providing excellent care.

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Section 2 - Electronic Case Reporting Pre-requisite Details

EHR Minimum Version

- Align with Regulatory Reporting minimum version requirements for the reporting year 2024.
- Prime Suite clients: Upgrade to 21.23.00.04 (21.23 HF4).
- Intergy clients: Upgrade to 21.25.00

Note - *You can view or download the Intergy 21.25.00 System Requirements document from the MyGreenway link: https://greenwayhealth.my.salesforce.com*

FHIR Opt-in

Opt-in to FHIR Services.

- https://records.greenwayhealth.com/

Remote Monitoring System (RMS)

- Communicating with client server successfully
- Connectivity required for EDH/FHIR Data fidelity with site's server

Direct Messaging

eCR clients must have direct messaging enabled at their site to enable the Electronic Case Reports to be sent to their Public Health Agencies (PHAs) and to receive reportability responses (RR) into the EHR.

Communicate Intent to Public Health Agency (PHA)

Practices can communicate their intent to start Electronic Case Reporting (eCR) to a local PHA. State and local PHAs support interoperability efforts and data exchange with electronic health records, many of which have been utilized by the Centers For Medicare and Medicaid Services (CMS) Promoting Interoperability Programs. To make eCR information of each state easily available, the Interoperability Standards Advisory (ISA) has put together a list of PHA websites (Appendix-IV) at the jurisdictional level. This list provides information about local implementation guides, programs that are supported, contact information, etc. on a statewide basis. This is a sort of hub that healthit.gov has put together in order to access each state's eCR info more easily

APHL Platform Onboarding

- Complete the eCR Facility List Excel Sheet which you can download from the MyGreenway link https://greenwayhealth.secure.force.com/util/sfly?g=rWKr49Ch9aEdZxUQBB4Yvky9v
- Pick a soft go live date in collaboration with their AIMS/CDC contact and Greenway Health.

APHL Policy Path

- Select a Policy Path.
- https://ecr.aimsplatform.org/healthcare/confirm-policy-path
- Recommended Policy Path: Opt-in to CommonWell.

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  - https://greenwayhealth.my.site.com/support/apex/CommunityCart?isdtp=mn&page=home
- Required to engage with APHL's AIMS platform for eCR.

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Section 3 - Regulatory Reporting Requirements

Regulatory Reporting Clients

Measure PI_PHCDRR_3:

As part of the 2024 MIPS rules for eCR, eligible clinicians are required to be actively engaged with a PHA to meet the eCR requirement (Measure PI_PHCDRR_3).

The 2024 MIPS eCR Requirements for the Promoting Interoperability "Public Health and Clinical Data Exchange" measure require eligible clinicians to be "Actively Engaged" in Electronic Case Reporting through one of two options, depending on adoption maturity. Greenway Health is actively developing an Electronic Case Reporting FHIR solution which will be available sometime in the 2024 performance period.

CMS has clarified that if eligible clinicians do not report "yes" for the eCR public health measure and provide their level of active engagement, they will receive a "0" score for their entire Promoting Interoperability (PI) category. The PI category represents 25% of the overall MIPS score. Clinicians may wish to adopt the certified health solution for Electronic Case Reporting that has been developed by Greenway Health.

Note - *In 2024, the Promoting Interoperability Performance Period is 180 consecutive days within the calendar year.*

The MIPS eligible clinician is in the process of moving towards sending "production data" to a PHA or Clinical Data Registry (CDR). Production data refers to data generated through clinical processes involving patient care, and it is used to distinguish between data and "test data" which may be submitted for the purposes of enrolling in and testing electronic data transfers.

Note - *You can access the latest version of the MIPS eCR Requirements from the qpp.cms.gov website.*

Active engagement may be demonstrated in one of the following ways:

Option 1 – Pre-Production and Validation:

The MIPS eligible clinician must first register to submit data with the PHA or, where applicable, the CDR to which the information is being submitted. Registration must be completed within 60 days after the start of the performance period, while awaiting an invitation from the PHA or CDR to begin testing and validation. MIPS eligible clinicians that have registered in previous years do not need to submit an additional registration for subsequent performance periods. Upon completion of the initial registration, the MIPS eligible clinician must begin the process of testing and validation of the electronic submission of data. The MIPS eligible clinician must respond to requests from the PHA or, where applicable, the CDR within 30 days; failure to respond twice within a performance period would result in the MIPS eligible clinician not meeting the measure.

Option 2 – Validation and Data Production:

The MIPS eligible clinician has completed testing and validation of the electronic submission and is electronically submitting production data to the PHA or CDR.

Merit-Based Incentive Payment System

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Merit-Based Incentive Payment System (MIPS) Promoting Interoperability Performance Category Measure 2024 Performance Period

Objective:	Public Health and Clinical Data Exchange
Measure:	Electronic Case Reporting The MIPS eligible clinician is in active engagement with a public health agency to electronically submit case reporting of reportable conditions.
Measure ID:	PI_PHCDRR_3
Exclusions:	Any MIPS eligible clinician meeting one or more of the following criteria may be excluded from the Electronic Case Reporting measure if the MIPS eligible clinician; 1. Does not treat or diagnose any reportable diseases for which data is collected by their jurisdiction's reportable disease system during the performance period. OR 2. Operates in a jurisdiction for which no public health agency is capable of receiving electronic case reporting data in the specific standards required to meet the certified electronic health record technology (CEHRT) definition at the start of the performance period. OR 3. Operates in a jurisdiction where no public health agency has declared readiness to receive electronic case reporting data as of 6 months prior to the start of the performance period.
Measure Exclusion IDs:	1. PI_PHCDRR_3_EX_1 2. PI_PHCDRR_3_EX_2 3. PI_PHCDRR_3_EX_3
Active Engagement Level IDs:	1. PI_PHCDRR_3_PRE 2. PI_PHCDRR_3_PROD

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Section 4 - Generation and Timing of Case Reports

Section 4.1 - Case Report Generation

- The eCR software of Greenway Health will handle the electronic case report creation and submission process.
- There are several specific pieces of data that will generate a case report, including Lab Orders, Problems, Diagnoses, Lab Results, Lab Result Test Names. For additional information on the triggering values and frequency of electronic case report submissions, visit: https://ecr.aimsplatform.org/ehr-implementers/triggering/
- Links to the eCR trigger codes and the trigger codes change log are as given below:
    - Reportable Condition Trigger Codes for ECR - Release 2024-04-05
    - Reportable Condition Trigger Codes for ECR - Change Log 2024-04-05
- Lab Orders can trigger case reports when a reportable LOINC code exists at the Lab Order level.
- Case reporting triggers are set up based on when the Encounter resource is first created.
- Checks for reportable data are based on the entered start (Onset) and end dates of the individual clinical data elements in the EHR system.
- When entering clinical data, the Onset Date must be entered. Items without an onset date may be excluded or may trigger a case report on outdated data.
- Newly entered clinical data with a status of "Entered in Error", "Unknown", or "Resolved" will not trigger a case report. If an earlier status is used when the data is first added (For example: “Active” -> “Resolved”), a case report may be triggered based on the previously reportable status.
- eCR reviews the status of case reports for immediate reporting 1 hour after data entry, and for standard reporting, 7 hours after data entry, 79 hours after data entry, and every 72 hours after that until the end of the reporting period (14 days). This ensures that PHAs receive the most up-to-date information for each Encounter.
- Data entered up to 14 days after the Encounter is included in case of reporting updates, even if the data is entered on a different Encounter in the same 14-day window. This is to ensure that clinical data coming in or entered after the Encounter (for example, Lab Results) is still captured and reportable, when needed.
- Encounters in a status of “In Progress” or “Finished” will be reviewed for trigger codes for up to 14 days following the Encounter.
- eCR will always check for “immediately reportable” triggers, regardless of the Encounter date.
- To generate a case report, the EHR user must be inside a patient Visit/Encounter when adding qualifying information to the patient's chart. Only information added during a Visit/Encounter can trigger an electronic case report.
- All Problems require a SNOMED to trigger for Electronic Case Reporting. Ensure that a SNOMED is added on your Problem and/or Diagnosis (if converting to a Problem).

Section 4.2 - Time limit for Case Report Generation

- Electronic case reporting relies on the use of data triggers and queries to confirm the most up-to-date information is included in a generated case report. For additional information on the triggering values and frequency of electronic case report submissions, visit: https://ecr.aimsplatform.org/ehr-implementers/triggering/.

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- The eCR software will review clinical data entered in the EHR from 3 days before to 14 days after an encounter for case reporting data "triggers". Data triggers would be any data entered into the EHR system that requires a case report to be generated and sent to the AIMS platform for ingestion by PHAs.

  Important Note - Some state regulations require that certain reportable conditions be reported in as little as an hour. If a Clinical Provider identifies an immediately reportable condition, the recommended practice is to contact your Public Health Agencies (PHA) as soon as possible. While the Electronic Initial Case Report (eICR) will be forthcoming, it is imperative that the Clinical Provider is able to share as much information as possible with the PHA prior to that.

  Note - *Date ranges are determined by review of the UTC time when the value was entered. In uncommon cases where data is entered near the beginning or end of a business day, case reports may trigger at 4 days before or 15 days after the Encounter start/end time. Generally, case reports will be created based on trigger data entered up to 3 days before and up to 14 days after the encounter starts.*

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Section 5 - eCR Technical Versions

The initial software release of Electronic Case Reporting (eCR) by Greenway (eCR v1.0) will include support for:

- Version 3 of the Smart-on-FHIR application "eCR Now".
- Version 2 of the Electronic Reporting and Surveillance Distribution (eRSD) specifications.
- Specification Bundle 1.1 - effective June 1st, 2023. Release 1.1 of the CDA standards for Electronic Initial Case Reports (eICR).

Note - *The Electronic Case Reporting solution by Greenway is currently pending certification to the* 170.315 *(f)(5) certification criteria.*

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Section 6 - Reportability Responses (RR)

The Reportability Response (RR) is a HL7 Clinical Document Architecture (CDA) standard for eCR that was developed to allow for communication back to healthcare from public health, which includes:

- The reportability status of conditions identified in an Electronic Initial Case Report (eICR).
- If there are reportable conditions, the jurisdiction(s) that were reported to, along with contact information for the PHAs.
- Information about reported conditions in the jurisdiction.
- Necessary follow-up testing or condition management information.
- Other relevant information (For example: Treatment guidelines, fact sheets, etc.).

A reportability response is created for each eICR received. While the RR is also shared with PHAs if they wish to receive it, the primary audience for the RR is the healthcare Provider/Reporter. The RR communicates information on the status of reportable conditions and providers next steps the healthcare provider may need to take relative to these conditions and their patients.

Remember - *Some state regulations require that certain reportable conditions be reported in as little as an hour. If a Clinical Provider identifies an immediately reportable condition, the recommended practice is to contact your Public Health Agencies (PHA) as soon as possible. While the Electronic Initial Case Report (eICR) will be forthcoming, it is imperative that the Clinical Provider is able to share as much information as possible with the PHA prior to that.*

Note –

- *When seeing a patient with an immediately-reportable issue, the Provider may still wish to contact their PHA directly, to ensure rapid response and outbreak measures are available.*
- *Providers can view Reportability Responses in the Document Import (Prime Suite) or Task Inbox (Intergy).*

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Reportability Response (RR) Example

Patient	Everywoman, Eve
Patient ID(s)	123453 2.16.840.1.113883.19.5 111-00-1234 2.16.840.1.113883.4.1
Contact info	Home: 2222 Home Street City, State 99999, US
Date of Birth	April 20, 1990
Sex	Female
Race	White
Ethnicity	Hispanic or Latino
Preferred Language	Portuguese
Primary Information Recipient:	Dr. Seven, Henry
Contact info	1002 Healthcare Drive City, State 99999, US
eICR Identifier:	ca316e79-aaf2-4e8c-aa5f-848093bb8bb1

Subject:

Public Health Reporting Communication: one or more conditions are reportable, or may be reportable, to public health.

Summary:

Your organization electronically submitted an initial case report to determine if reporting to public health is needed for a patient. "Zika virus disease (disorder)" is reportable to "State Department of Health".

An initial case report was sent to "State Department of Health". Additional information may be required for this report.

"Zika virus disease (disorder)" for "State Department of Health"

Reporting is required within "24 hours". Reporting to this Public Health Agency is based on "both patient home address and provider facility address".

- Local mosquito-borne Zika virus transmission was reported in your area. Increased watchfulness for symptoms of Zika virus in your patients is warranted. (Immediate action requested)
- Additional information for the required reporting of Zika must be submitted to State Department of Health immediately. This additional information can be submitted here. (Link - Immediate action required)
- Zika has particular risks for pregnant women. Follow-up guidance for pregnant women and couples who are planning pregnancy. (Link - Immediate action requested)
- Further Laboratory testing for Zika may be needed. Guidance for additional testing and specimen collection (Link - Action requested)
- Forms for submitting further Zika laboratory testing (Link - Information only)
- Treatment guidance (Link - Information only)
- If you have additional questions regarding Zika or reporting, contact information for the State Department of Health is available here. (Link - Information Only)

Additional Resources (Information only):

- Control and prevention information for providers (Link)
- Detailed condition references (Link)
- Prevalence in State (Link)
- CDC webpage (Link)
- Patient information factsheet (Link)
- State Resident information (Link)

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